EVERY VOICE DESERVES TO BE HEARD.

A proposal to hospital leadership, and to the Association of Sign Language Interpreters of Nigeria — the room and the language, addressed together.

01 THE GAP

Interpreter provision in Nigerian hospitals is rare, unscheduled and expensive. In 2024 we visited 21 hospitals to ask who interprets when a Deaf patient arrives.

The answer was consistent: a relative, another patient, or a clinician improvising gestures. The conversation still happens. It just is not safe.

THE GAP IN NUMBERS

Unmute field visits, 21 Nigerian hospitals, 2024.

21
HOSPITALS VISITED

0
HAD AN INTERPRETER ON SITE

16
SIGNED LETTERS OF INTENT

WHAT WE ARE NOT CLAIMING

There is no validated Nigerian Sign Language model yet. Unmute begins with ASL and routine interactions. A pilot is an evaluation, not a deployment.

This document is a proposal to ASLIN. It implies no partnership, endorsement or agreement until one is signed.

02 WHAT UNMUTE IS

01 THE PATIENT SIGNS
Supported sign language is turned into text and speech the clinician can read or hear immediately.

02 THE CLINICIAN REPLIES
A spoken or typed reply becomes visual sign-language output on the same screen, so the loop closes in the room.

03 ROUTINE, NOT EVERYTHING
Built around a predefined set of routine clinical interactions — pain, history, consent, next steps — so what it says can be reviewed before it is trusted.

03 WHAT WE ARE ASKING

FROM HOSPITALS

A named clinical lead, one department, up to ten practitioners, and one month of routine use. We install, we train, we measure accuracy and usability with you. No charge for the pilot.

FROM ASLIN

A working session on the signs Nigerian patients and clinicians actually reach for, including regional and local variants. We fund the sessions, credit every contributor, and treat Nigerian Sign Language as a language in its own right — not a dialect of ASL.

04 WHY ONE DOCUMENT

A hospital can install software, but it cannot decide which signs a Deaf patient uses. Neither half of the gap closes on its own.

THE PILOT, IN OUTLINE

SETUP
Half a day on site: install, account setup, and training for the team.

SCOPE
One department, up to ten practitioners, routine clinical interactions.

DURATION
Four weeks of ordinary use — not a scripted demonstration.

MEASURE
Recognition accuracy, time saved, and where it fails. Reported to you.

EXIT
No obligation. If it does not earn its place, we stop and say so.

NEXT STEP One visit, thirty minutes, at your hospital.

Bring the clinical lead — and whoever already interprets for your Deaf patients.

Unmute

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